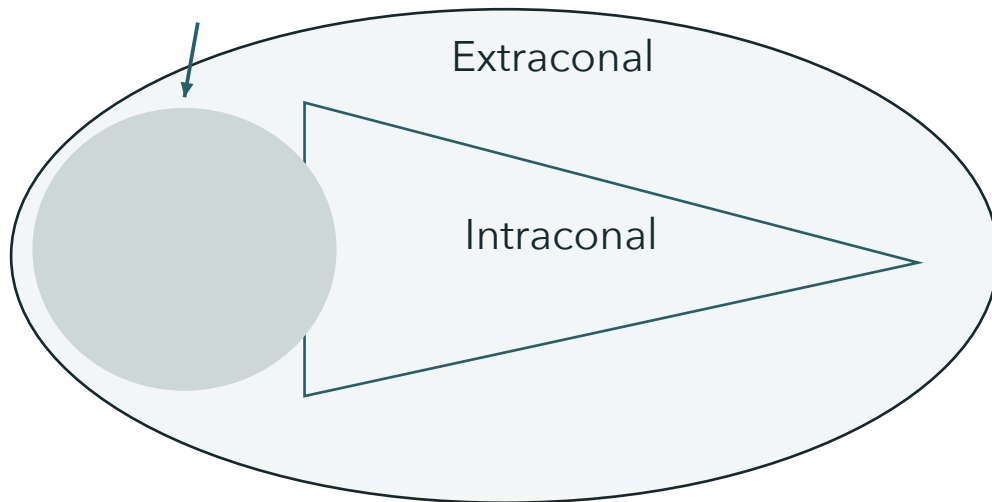


Orbit lids and lacrimal selection

Parts 1 and 2 | Compartment and tissue behaviour determine the approach

Plan the route before selecting biopsy or excision.

Lacrimal fossa



Original orbital map: compartment and displacement guide the tissue strategy.

A slowly growing, well-circumscribed lacrimal-gland mass compatible with pleomorphic adenoma favours planned intact removal rather than an indiscriminate incisional biopsy. Suspected orbital lymphoma requires tissue diagnosis with an appropriate specimen pathway. An incidental stable cavernous venous malformation without visual compromise can be observed. Anatomical location alone therefore does not dictate one tissue strategy.

Thyroid orbitopathy: identify the current target

Activity, severity and sight threat should be stated separately.

Finding	Implication
Inflammatory active disease	Consider appropriate systemic treatment context
Optic threat	Urgent response and decompression assessment
Stable restrictive diplopia	Plan binocular rehabilitation after relevant preceding changes
Targeted drugs	Discuss current label and extraocular toxicity

Active moderate-to-severe thyroid eye disease and stable restrictive diplopia require different treatment targets. Systemic treatment addresses inflammation; definitive strabismus repair generally follows disease stability and any required decompression. Sight-threatening optic neuropathy needs urgent escalation. Teprotumumab selection also requires counselling and monitoring for hearing impairment, hyperglycaemia and inflammatory bowel disease in its current label context.

Lid repair should change the measured force

A diagnostic label does not describe every mechanical component.

Finding	Implication
Poor levator action	Consider a strategy that bypasses weak elevation
Horizontal lower-lid laxity	Restore appropriate support
Retractor failure	Address the vertical stabilising component
Scar-related shortage	Replace deficient tissue length when indicated

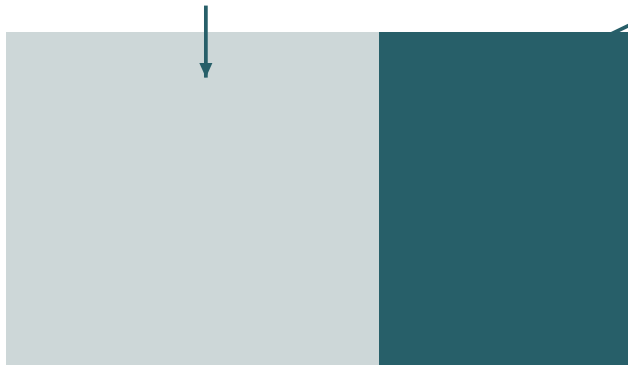
Severe ptosis with poor levator action may require frontalis suspension to bypass the weak elevator; levator advancement better fits an aponeurotic mechanism with preserved action. Involutional entropion often combines horizontal laxity and retractor dysfunction, whereas cicatricial malposition adds tissue shortage. A repair that corrects only one demonstrated component may leave recurrence or corneal exposure.

Reconstruction needs a living bed and a safe surface

Restore lining, support and closure together.

Anterior lamella
Skin + orbicularis

Posterior lamella
Tarsus + conjunctiva



A free graft requires a vascularised bed.

Original section schematic: restore lining, support, movement and margin continuity.

Original lamellar section: a vascularised component supports an appropriate free graft.

A full-thickness defect requires anterior covering and posterior support, with a vascularised component sustaining an opposing free graft. Restore margin continuity, closure and a smooth corneal-facing surface. Two free grafts without a suitable vascular bed risk failure. Check function as well as appearance when assessing the repair.

Lacrimal bypass: anatomy and function are separate

A route under pressure need not clear tears during blinking.

Finding	Implication
Canalicular level	Stop and reflux pattern help localisation
Usable canaliculi with distal obstruction	Consider a sac-to-nose bypass
Unreconstructable proximal obstruction	Consider tear-lake-to-nose bypass
Patent operation but persistent symptoms	Reassess entry, pump and surface

A hard stop demonstrates entry to the sac, while the reflux and nasal passage pattern further localise obstruction. Distal obstruction with usable canaliculi can suit DCR; unreconstructable proximal obstruction may require a Jones-tube bypass. Persistent watering despite a patent DCR requires assessment of punctal entry, blink pump and ocular surface, rather than automatically repeating the same operation.